GOVCARE EHR

CKD AI CLINICAL DECISION SUPPORT REPORT

Clinical decision support only - clinician review required

PATIENT & REPORT DETAILS

Patient	Demo Patient H02 0241
Patient number	PAT-NET-H02-0241
DOB	1948-10-09
Sex	male
Assessment ID	6bea788f-eee2-4259-9377-26c6ee744b69
Assessment time	2026-10-12 11:25:01.542728
Hospital	GovCare Demo Hospital 02
Department	Not recorded
Clinician / operator	Local Super Admin
Model	GOVCARE-CKD-LOGISTIC-1.0

CKD RISK SUMMARY

Predicted CKD risk	Risk category	AI confidence
96.9%	**URGENT REVIEW**	**93.9%**

CKD CLASSIFICATION & CLINICAL CODING

Suggested CKD classification	Stage 2
G category	G2
Suggested ICD code	N18.2
Clinical description	Chronic kidney disease, stage 2
Terminology system	http://hl7.org/fhir/sid/icd-10-cm
Basis	Reviewed eGFR 72 mL/min/1.73m²; chronicity and diagnosis require clinician confirmation
Coding status	AI_ASSISTED_SUGGESTION
Confirmation	Clinician confirmation required

A single eGFR result does not independently establish chronic kidney disease or chronicity. N18.6 (ESRD) is not inferred from eGFR alone.

○ G1	● G2	○ G3a	○ G3b	○ G4	○ G5

RENAL & ASSOCIATED FINDINGS

Parameter	Result	Unit	Source
eGFR	72	mL/min/1.73m²	PDF 02_partial_ckd_text_report.pdf 95% confidence
Serum creatinine	1.403	mg/dL	PDF 03_unit_conversion_and_unrecognized_report.pdf 95% confidence

Urine albumin/creatinine ratio (ACR)	55	mg/g	PDF 03_unit_conversion_and_unrecognized_report.pdf 95% confidence
Urine protein	12	mg/dL	MANUAL
Systolic blood pressure	146	mmHg	PDF 01_full_ckd_text_report.pdf 95% confidence
Diastolic blood pressure	92	mmHg	PDF 01_full_ckd_text_report.pdf 95% confidence
HbA1c	6.4	%	PDF 02_partial_ckd_text_report.pdf 95% confidence
BMI	27.4	kg/m²	PDF 01_full_ckd_text_report.pdf 95% confidence
Serum potassium	1	mmol/L	MANUAL
Hemoglobin	13.8	g/dL	PDF 05_no_ckd_values_report.pdf 95% confidence

CKD-RELATED CLINICAL RISKS & PREDICTIVE INTELLIGENCE

Sri Lankan Longitudinal Renal & Hemodialysis Data

Saved snapshot: c14770ab-89d8-4606-8d30-8aed95d22dc3 | Captured 2026-10-12T05:55:01.714567200Z. Export reads stored outputs; no inference is performed.

Renal Clinical Trajectory

Result not calculated - insufficient longitudinal data.

Method: Longitudinal Trend

Result not ready yet

0 of 6 domains currently evaluable. Each domain needs 2 valid dated results; 1 evaluable domain required for this result.

Needs: Creatinine (0/2)

Needs: Urea (0/2)

Needs: Potassium (0/2)

Creatinine: 0/2 valid dated results; more source data required

Urea: 0/2 valid dated results; more source data required

Potassium: 0/2 valid dated results; more source data required

Hemoglobin: 0/2 valid dated results; more source data required

Albumin: 0/2 valid dated results; more source data required

URR: 0/2 valid dated results; more source data required

Clinical rule | LK-RENAL-MODULES-1.1 | NOT CLINICALLY VALIDATED

Review: AWAITING REVIEW | version 0

Monitoring / research support; clinician interpretation required. Full readiness and limitations follow.

Kidney Failure / G5D

Result not calculated - insufficient longitudinal data.

Method: Clinical Rule

Status not established

Current eGFR alone does not establish chronic kidney failure. Verify chronic dialysis and clinical context; high creatinine cannot establish G5D.

Needs: Chronic dialysis status (0/1)

Needs: Dialysis verification date (0/1)

Needs: CKD / chronicity context (0/1)

Clinical evidence: Chronicity: NOT_ESTABLISHED; Categories describe recorded measurements. CKD diagnosis requires clinician confirmation, chronicity and clinical context; G1/G2 alone do not establish CKD.; current eGFR-based clinical assistance, chronicity and diagnosis require clinician confirmation. | Verified: Not recorded

Clinical rule | LK-RENAL-MODULES-1.1 | NOT CLINICALLY VALIDATED

Review: AWAITING REVIEW | version 0

Monitoring / research support; clinician interpretation required. Full readiness and limitations follow.

Hyperkalemia Monitoring

Eligible dialysis context not verified; model not applicable under current eligibility criteria.

Method: Longitudinal Research Model

Eligible dialysis context not verified

This research model applies to the supported maintenance-hemodialysis cohort. The clinical record does not verify eligible chronic dialysis; eligibility is not established.

Needs: Verified maintenance hemodialysis (0/1)

LK-RENAL-HYPERK-1.0 | 1.0.0 | INTERNALLY VALIDATED

Review: AWAITING REVIEW | version 0

Monitoring / research support; clinician interpretation required. Full readiness and limitations follow.

Anemia / Hb Deterioration

Eligible dialysis context not verified; model not applicable under current eligibility criteria.

Method: Persistence Baseline

Eligible dialysis context not verified

This research model applies to the supported maintenance-hemodialysis cohort. The clinical record does not verify eligible chronic dialysis; eligibility is not established.

Needs: Verified maintenance hemodialysis (0/1)

LK-RENAL-HB-1.0 | 1.0.0 | INTERNALLY VALIDATED

Review: AWAITING REVIEW | version 0

Monitoring / research support; clinician interpretation required. Full readiness and limitations follow.

Dialysis Adequacy / URR

Eligible dialysis context not verified; model not applicable under current eligibility criteria.

Method: Longitudinal Research Model

Eligible dialysis context not verified

This research model applies to the supported maintenance-hemodialysis cohort. The clinical record does not verify eligible chronic dialysis; eligibility is not established.

Needs: Verified maintenance hemodialysis (0/1)

Current measured URR: Unavailable% | Next-period forecast: Not ready / not run

URR = ((Pre-HD Urea - Post-HD Urea) / Pre-HD Urea) x 100

LK-RENAL-URR-1.0 | 1.0.0 | INTERNALLY VALIDATED

Review: AWAITING REVIEW | version 0

Monitoring / research support; clinician interpretation required. Full readiness and limitations follow.

Renal Clinical Deterioration

Result not calculated - insufficient longitudinal data.

Method: Transparent Composite

Not enough longitudinal data

0 of 6 domains currently evaluable. Each domain needs 2 valid dated results; 3 evaluable domains required for this result.

Needs: Creatinine (0/2)

Needs: Urea (0/2)

Needs: Potassium (0/2)

Creatinine: 0/2 valid dated results; more source data required

Urea: 0/2 valid dated results; more source data required

Potassium: 0/2 valid dated results; more source data required

Hemoglobin: 0/2 valid dated results; more source data required

Albumin: 0/2 valid dated results; more source data required

URR: 0/2 valid dated results; more source data required

Clinical rule | LK-RENAL-MODULES-1.1 | NOT CLINICALLY VALIDATED

Review: AWAITING REVIEW | version 0

Monitoring / research support; clinician interpretation required. Full readiness and limitations follow.

Clinical Decision Support / Research Use. Outputs require clinician interpretation. Research-model outputs are not independently validated diagnoses.

SIX-MODULE EVIDENCE & CLINICIAN REVIEW

Renal Clinical Trajectory

Result not calculated - insufficient longitudinal data.

Needs review / data: Creatinine | 0/2 | No recorded result; need 2 valid dated results
Needs review / data: Urea | 0/2 | No recorded result; need 2 valid dated results
Needs review / data: Potassium | 0/2 | No recorded result; need 2 valid dated results
Needs review / data: Hemoglobin | 0/2 | No recorded result; need 2 valid dated results
Needs review / data: Albumin | 0/2 | No recorded result; need 2 valid dated results
Needs review / data: URR | 0/2 | No recorded result; need 2 valid dated results

Next action: Add missing renal labs

Directions compare the two latest valid values per domain using a 1% neutral band; this is not a prognosis or a G1-to-G5 forecast.
Dialysis timing, clinical context and measurement uncertainty require clinician interpretation.

Source measurements: 0 | Reliability: INSUFFICIENT | Horizon: No probability horizon | Last run: Not recorded

Kidney Failure / G5D

Result not calculated - insufficient longitudinal data.

Needs review / data: Chronic dialysis status | 0/1 | Chronic dialysis context is not established
Needs review / data: Dialysis verification date | 0/1 | 0/1 available
Needs review / data: CKD / chronicity context | 0/1 | Chronicity / diagnosis requires clinician confirmation
Available: Latest valid eGFR | 0/1 | 0/1 available

Next action: Verify dialysis status

High creatinine alone does not establish G5D. This card reports recorded clinical evidence, not a diagnosis generated by AI.
No 2-year or 5-year kidney-failure probability is calculated.

Source measurements: 0 | Reliability: INSUFFICIENT | Horizon: No probability horizon | Last run: Not recorded

Hyperkalemia Monitoring

Eligible dialysis context not verified; model not applicable under current eligibility criteria.

Available: Reviewed model and metadata | 1/1 | Reviewed artifact and serving input contract verified
Needs review / data: Verified maintenance hemodialysis | 0/1 | 0/1 available

Next action: Verify dialysis status

Research outputs require clinician interpretation; internal evaluation is not independent clinical validation.
Dataset permission status remains UNVERIFIED.
This installed model is scoped to verified maintenance hemodialysis. No applicable dialysis context is recorded.

Requires two fresh verified HD observations 14–62 days apart, a current target result and five supported predictor values.
Source measurements: 0 | Reliability: INSUFFICIENT | Horizon: No probability horizon | Last run: Not recorded

Anemia / Hb Deterioration

Eligible dialysis context not verified; model not applicable under current eligibility criteria.

Available: Reviewed model and metadata | 1/1 | Reviewed artifact and serving input contract verified
Needs review / data: Verified maintenance hemodialysis | 0/1 | 0/1 available

Next action: Verify dialysis status

Research outputs require clinician interpretation; internal evaluation is not independent clinical validation.
Dataset permission status remains UNVERIFIED.
Learned candidates failed the gate against persistence. Carries forward the last measured Hb; no anemia-classifier probability is installed.
This installed model is scoped to verified maintenance hemodialysis. No applicable dialysis context is recorded.

Requires two fresh verified HD observations 14–62 days apart, a current target result and five supported predictor values.
Source measurements: 0 | Reliability: INSUFFICIENT | Horizon: No probability horizon | Last run: Not recorded

Dialysis Adequacy / URR

Eligible dialysis context not verified; model not applicable under current eligibility criteria.

Available: Reviewed model and metadata | 1/1 | Reviewed artifact and serving input contract verified
Needs review / data: Verified maintenance hemodialysis | 0/1 | 0/1 available

Next action: Verify dialysis status

Research outputs require clinician interpretation; internal evaluation is not independent clinical validation.
Dataset permission status remains UNVERIFIED.
This installed model is scoped to verified maintenance hemodialysis. No applicable dialysis context is recorded.

Requires two fresh verified HD observations 14–62 days apart, a current target result and five supported predictor values.
Source measurements: 0 | Reliability: INSUFFICIENT | Horizon: No probability horizon | Last run: Not recorded

Renal Clinical Deterioration

Result not calculated - insufficient longitudinal data.

Needs review / data: Creatinine | 0/2 | No recorded result; need 2 valid dated results

Needs review / data: Urea | 0/2 | No recorded result; need 2 valid dated results
Needs review / data: Potassium | 0/2 | No recorded result; need 2 valid dated results
Needs review / data: Hemoglobin | 0/2 | No recorded result; need 2 valid dated results
Needs review / data: Albumin | 0/2 | No recorded result; need 2 valid dated results
Needs review / data: URR | 0/2 | No recorded result; need 2 valid dated results

Next action: Complete renal trend data

Directional composite – not a validated prognosis score.

Equal weights; two latest valid observations per domain; 1% neutral stability band. Missing domains are excluded, never scored as zero. A directional percentage is not a disease probability.

Source measurements: 0 | Reliability: INSUFFICIENT | Horizon: No probability horizon | Last run: Not recorded

PREDICTIVE CLINICAL INTELLIGENCE - LONGITUDINAL REVIEW

Live authorized EHR history at report generation time. Projected values are statistical extrapolations and are never stored or presented as laboratory observations.

Recorded G / A category	Latest eGFR	Latest creatinine	Data completeness
- / -	Unavailable	Unavailable	Unavailable

HISTORICAL eGFR & PROJECTED TRAJECTORY

No usable dated eGFR history is available.

Horizon	Current	Projected	Projected range	Trend	Reliability	Status / interval method
3 months	-	Unavailable	Not estimated	UNAVAILABLE	LOW	INSUFFICIENT DATA NOT ESTIMATED
6 months	-	Unavailable	Not estimated	UNAVAILABLE	LOW	INSUFFICIENT DATA NOT ESTIMATED
12 months	-	Unavailable	Not estimated	UNAVAILABLE	LOW	INSUFFICIENT DATA NOT ESTIMATED

EXPLAINABILITY & DATA-QUALITY LIMITATIONS

Renal safety: INSUFFICIENT DATA. Renal observations have unresolved date, unit, provenance, plausibility or conflicting-result issues. Reconcile the source records before forecasting. A usable eGFR result from the previous 90 days is unavailable. A usable creatinine result from the previous 90 days is unavailable. At least two dated eGFR measurements spanning 90 days are required by the longitudinal sufficiency policy. An active or recent admission requires acute illness and fluid-status review before long-term forecasting (GovCare 14-day context policy).

- MISSING CLINICAL DATE: A usable measurement date is required; upload time is never substituted.
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Medication context: No recent active prescription record was found. Medication use and nephrotoxic exposure are unknown; reconcile with the patient.

CLINICIAN-CONTROLLED RECOMMENDATIONS

- No dated eGFR available | AWAITING REVIEW | Confirm whether a newer result exists and whether repeat measurement is clinically appropriate.
- No dated Urine ACR available | AWAITING REVIEW | Confirm whether a newer result exists and whether repeat measurement is clinically appropriate.

Method: TREND PROJECTION · Model: trend-linear-2.0 · Validation: STATISTICAL_PROJECTION_NOT_CLINICALLY_VALIDATED · Local validation: NOT_APPLICABLE

Clinical decision support only. Forecasts and risk estimates are not autonomous diagnoses.

CLINICAL DATA USED FOR ASSESSMENT

Parameter	Result	Unit	Source	Coding

Urine albumin/creatinine ratio (ACR)	55	mg/g	PDF 03_unit_conversion_and_unrecognized_report.pdf 95% confidence	-
Age	78	-	CALCULATED	-
BMI	27.4	kg/m²	PDF 01_full_ckd_text_report.pdf 95% confidence	LOINC 39156-5
Blood urea nitrogen (BUN)	12	mg/dL	MANUAL	-
Diastolic blood pressure	92	mmHg	PDF 01_full_ckd_text_report.pdf 95% confidence	LOINC 8462-4
Edema	0	-	MANUAL	-
Family history of kidney disease	0	-	MANUAL	-
Fasting blood sugar	324	mg/dL	MANUAL	-
eGFR	72	mL/min/1.73m²	PDF 02_partial_ckd_text_report.pdf 95% confidence	LOINC 48642-3
HbA1c	6.4	%	PDF 02_partial_ckd_text_report.pdf 95% confidence	LOINC 4548-4
Hemoglobin	13.8	g/dL	PDF 05_no_ckd_values_report.pdf 95% confidence	LOINC 718-7
NSAID use	0	-	MANUAL	-
Previous acute kidney injury	0	-	MANUAL	-
Urine protein	12	mg/dL	MANUAL	-
Serum creatinine	1.403	mg/dL	PDF 03_unit_conversion_and_unrecognized_report.pdf 95% confidence	LOINC 2160-0
Serum potassium	1	mmol/L	MANUAL	-
Systolic blood pressure	146	mmHg	PDF 01_full_ckd_text_report.pdf 95% confidence	LOINC 8480-6

KEY FACTORS IN THIS MODEL OUTPUT

Contribution direction describes this model's prediction, not whether a clinical value is healthy, safe or protective.

Factor	Value	Unit	Direction	Contribution
Urine protein	12	mg/dL	Increases prediction	2.6469
Fasting blood sugar	324	mg/dL	Increases prediction	1.9130
Serum creatinine	1.403	mg/dL	Reduces prediction	-0.8201
Serum potassium	1	mmol/L	Reduces prediction	-0.5030
Blood urea nitrogen (BUN)	12	mg/dL	Reduces prediction	-0.4512
Systolic blood pressure	146	mmHg	Increases prediction	0.1761
Edema	0	-	Reduces prediction	-0.1439
Urine albumin/creatinine ratio (ACR)	55	mg/g	Increases prediction	0.1296

RECOMMENDED CLINICAL REVIEW

1. Prompt clinician review is recommended.
2. Correlate with current renal function, urine findings, and clinical examination.

CLINICIAN REVIEW

Suggested classification	Stage 2
Suggested code	N18.2
Coding decision	[ ] Confirm [ ] Modify [ ] Reject
Final clinician-selected code	_____
Clinical notes	_____ _____
Reviewed by / date	_____

This output does not diagnose disease or replace professional medical judgement. AI-assisted coding must be reviewed before a confirmed FHIR Condition is created.